

Service Specification

Service	Provision of medical cover for Intermediate Care Pathway 2 patients – Northern, Eastern and West Localities including the UHP facing South Hams GP Practices
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Provider Lead	
Period	1 st April 2025 – 31 st March 2026
Date of Review	

1. Population Needs

1.1 National/local context and evidence base

All providers have detailed discharge planning processes in place which facilitate the smooth flow of patients from hospital to their usual place of residence.

This includes a range of options for people who need some additional support on discharge or need to have their care continued in an alternative setting. Alternative options include, rehabilitation units, community hospitals, nursing and residential homes and then home with community services including domiciliary care and home based reablement.

The principles of discharge are based on national guidance which is known as 'discharge to assess', which essentially describes a model whereby assessments are not completed in hospital but in the person's own home or next placement.

The discharge to assess model is based on using four clear pathways for discharging patients as shown below:

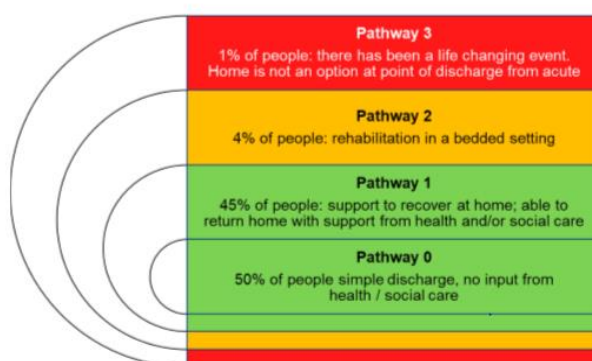


Figure 1: Discharge to Assess model

This provision could be within existing residential / nursing care beds or new sites which have been identified as intermediate care pathway 2 beds, providing rehabilitation and reablement in a bedded setting.

The service needs to ensure sufficient primary medical cover for the people being discharged from hospital into the identified intermediate care pathway 2 beds.

It is not the intention to use this provision for patients who are planning to be long term clients in residential or nursing care homes. intermediate care pathway 2 clients are expected to be short-term placements with an estimated average length of stay of 28 days.

The NHS England Intermediate care framework for rehabilitation, reablement and recovery following hospital discharge states:

<https://www.england.nhs.uk/publication/intermediate-care-framework-for-rehabilitation-reablement-and-recovery-following-hospital-discharge/>

The Intermediate care framework “is focused on step-down intermediate care – time-limited, short-term (typically no longer than 6 weeks) health and/or social care provided to adults (aged 18 years or over) who need support after discharge from acute inpatient settings and virtual wards to help them rehabilitate, re-able and recover.

Implementation of step-down intermediate care as outlined here is expected to result in improved outcomes, experiences and independence of people discharged, reduced avoidable hospital readmissions, and reduced avoidable/premature long-term care provision. Further expected benefits include improved flow and discharge from acute and community hospitals, freeing-up NHS hospital capacity for those who need it most.

Step-down intermediate care involves short-term community bedded settings (bed-based; discharge pathway 2).

Intermediate care services can be entirely health care, entirely social care, or ideally have elements of both delivered by multi-disciplinary teams working in integrated ways. They may be commissioned by the NHS, and/or by local authorities, and delivered by a range of providers across the health, social care, housing, independent and voluntary sectors.

Appropriate capacity should be commissioned (within available budgets) for people with step-down intermediate care needs, including people with particular conditions, needs, protected characteristics, or those living in particular circumstances, to prevent exclusion or sub-optimal care. This includes (but is not limited to): people living with frailty, mild cognitive impairment, dementia or delirium, palliative and end of life care needs, mental health conditions, learning disabilities, autism, obesity (including bariatric patients), younger adults, and people living in an unsafe environment, in prison, experiencing homelessness, or at risk of homelessness.

This work sits within the context of the wider effort to improve discharge planning, in-hospital flow, implementation of virtual wards, and other related initiatives.

2. Outcomes

2.1 NHS Outcomes Framework Domains & Indicators

Domain 1	Preventing People from dying prematurely	
Domain 2	Enhancing quality of life for people with long-term conditions	
Domain 3	Helping people to recover from episodes of ill health or following injury	✓
Domain 4	Ensuring people have a positive experience of care	✓
Domain 5	Treating and caring for people in safe environment and protecting them from avoidable harm	✓

3. Scope

3.1 Aims and objectives of service

Provision of primary medical cover for all intermediate care pathway 2 patients placed in residential, nursing care homes and dedicated rehabilitation and recovery beds.

3.2 Service description/care pathway

If patients discharged on an intermediate care Pathway 2 placement are placed in a care home outside of their registered GP area, they will be registered with a named GP practice as temporary residents until they are fit to be discharged from the intermediate care Pathway 2 facility. They will be able to access core primary medical services provided by that practice.

3.3 Population covered

All patients that are discharged to a care home on a discharge to assess intermediate care Pathway 2 bed in Northern, Eastern and West Localities including the UHP facing South Hams GP Practices (*a list of GP practices which this LES is applicable to can be found in the appendix. Where practices within the NHS Devon ICB footprint are not listed, alternative commissioning arrangements are in place*).

3.4 Any acceptance and exclusion criteria and thresholds

- Practices will provide this primary medical service to patients who are placed in an intermediate care Pathway 2 bed taking into account Care Home aligned with the PCN and the catchment area of the practice
- Patients who are not identified as being on discharge to assess intermediate care Pathway 2 placement
- Medical cover is not provided out of hours times (Mon – Fri 18:30 – 08:00 and weekends and bank holidays)
- As these patients are expected to be in the care home on a short-term temporary basis and if registered as a temporary resident they will not be subject to QOF or IIF requirements

3.5 Interdependence with other services/providers

Discharge summaries are expected to be provided to the patient's registered GP practice and the practice providing this intermediate care P2 service (if applicable) in a timely manner using currently agreed communication practice. Should it be required, this should be replaced by or supplemented by an email to the practice providing this intermediate care P2 service (e.g. in the case of a patient receiving this service as a temporary resident where the hospital system does not allow for sending to the practice of a temporary resident).

3.5.1 Wrap around support

In developing this model, the practice retains responsibility for the medical support, but it is acknowledged that it is essential that a model is developed to ensure that general practice is supported. The additional wrap around support can vary location to location but could include:

- Support from consultant teams
- District nursing and other community services
- Rapid response services
- Community and acute speciality advisory services.
- Community equipment
- Volunteers

The practice providing primary care medical cover will be expected to agree with the managers of the facility the method of interaction to enable a good working relationship with the home and the residents.

3.5.3 Out of Hours provision

Usual out of hours arrangements for general practice will apply.

4. Applicable Service Standards

4.1 The General Medical Services (GMS) Contract November 2022 state:

Temporary residents

13.6.1. The Contractor may, if the Contractor's list of patients is open, accept a person as a temporary resident provided the Contractor is satisfied that the person is:

(a) temporarily resident away from their normal place of residence and is not being provided with essential services (or their equivalent) under any other arrangement in the locality where that person is temporarily residing; or

(b) moving from place to place and not for the time being resident in any place

13.6.2. For the purposes of clause 13.6.1, a person is to be regarded as temporarily resident in a place if, when that person arrives in that place, they intend to stay there for more than 24 hours but not more than three months.

13.6.3. Where the Contractor wants to terminate its responsibility for a person accepted as a temporary resident before the end of:

(a) three months; or

(b) such shorter period for which the Contractor agreed to accept that person as a patient,

the Contractor must give notice of that fact to the person either orally or in writing and the Contractor's responsibility for that person is to cease seven days after the date on which notice is given.

13.6.4. Where the Contractor's responsibility for a person as a temporary resident comes to an end, the Contractor must give notice in writing to the Board of its acceptance of that person as a temporary resident:

(a) at the end of the period of three months beginning with the date on which the Contractor accepted that person as a temporary resident, or

(b) If the Contractor's responsibility for that person as a temporary resident came to an end earlier than at the end of the three month period referred to in sub-clause (a), at the end of that period.

5. Applicable quality requirements and CQUIN goals

Not applicable

6. Location of Provider Premises

The individual practice at which a patient is registered.
Care is provided at the address given on the Temporary Resident registration form.

7. Finance

7.1 Payment

Payment for registering temporary residents is included in the global sum. However, it is recognised that intermediate care Pathway 2 patients may require increased general practice provision.

The rate paid per patient on a discharge to assess intermediate care Pathway 2 bed will be £200. This has assumed of an average 28-day length of stay (taken from data collected recently).

A review of inflationary uplift will be undertaken on an annual basis. This will include the need for the service, level of activity and consider DDRB in the process.

Payment will be made to the practice based on data provided to NHS Devon via the Commissioning Intelligence Portal

All financial claims for 2025/26 are to be made no later than 30 April 2026.

7.2 Data capture

It will be essential that sufficient records are kept to:

- Meet the needs for payment to the practice.
- Ensure that patient records are kept accurately.

If the patient is a temporary resident the practice provides a discharge summary for the permanent practice (may be a note in shared record) in a timely way and in line with the usual process.

Appendix	Pathway 2 LES applicable to GP Practices	
Locality	PCN	Practice
East	Culm Valley	Blackdown Practice
		Bramblehaies Surgery
		College Surgery Partnership
		Sampford Peverell Surgery
		Wyndham House Surgery
East	Exeter City	Barnfield Hill Surgery
		Clock Tower Surgery (Inclusion Health Devon)
		Southernhay House Surgery
		St Leonards Practice
		Whipton Surgery
		Wonford Green Surgery
East	Exeter West	Foxhayes Practice
		St Thomas Medical Group
East	HOSMS	Coleridge Medical Centre
		Honiton Surgery
		Sid Valley Practice
East	Mid Devon HC	Bow Medical Practice
		Redlands Primary Care
		Wallingbrook Health Group
East	NEXUS	Heavitree Practice
		Hill Barton Surgery
		Isca Medical Practice
		Mount Pleasant Health Centre
		South Lawn Medical Practice
East	North Dartmoor	Blake House Surgery
		Chagford Health Centre
		Cheriton Bishop Surgery
		Moretonhampstead Health Centre
		Okehampton Medical Centre
East	Outer Exeter	Cranbrook Medical Practice
		Ide Lane Surgery
		Pinhoe Surgery
		Topsham Surgery
		Westbank Practice
East	TASC	Axminster Medical Practice
		Seaton & Colyton Medical Practice
		Townsend House Medical Centre

East	Tiverton	Amicus Health
		Castle Place Practice
		Mid Devon Medical Practice
East	WEB	Budleigh Salterton Medical Practice
		Haldon House Surgery
		Imperial Surgery
		Rolle Medical Partnership
		Woodbury Surgery
North	Barnstaple Alliance	Brannam Medical Centre
		Fremington Medical Centre
		Litchdon Medical Centre
		Queens Medical Centre
North	Coast & Country	Ruby Country Medical Group
		Bradworthy Surgery
North	North Devon Coastal	Caen Medical Centre
		South Molton Medical Centre
		Combe Coastal Practice
North	Torridge	Bideford Medical Centre
		Castle Gardens Surgery
		Hartland Surgery
		Northam Surgery
		Wooda Surgery
		Torrington Health Centre
South	South Dartmoor and Totnes	South Brent Health Centre
South	South Hams	Chillington Health Centre
		Dartmouth Medical Practice
		Modbury Health Centre
		Norton Brook Medical Centre
		Redfern Health Centre
West	Beacon	Beacon Medical Group
West	Mewstone	Wembury Surgery
		Yealm Medical Centre
West	West Devon	Abbey Surgery
		Tavyside Health Centre
		Yelverton Surgery